



P04 · 24 PAGES · FOR THE PARTNER

The Partner's Postpartum Playbook

Twenty-four pages, written directly to the non-birthing partner. Daily action cards for the first six weeks. A weekly support guide. The gift she'd actually want.

Soothemade **Notes**

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY



Read this first

This guide is for you — the partner of someone who just had a baby (or is about to).

Here's what most "how to help your wife" articles get wrong: they're written to her, telling her how to ask. By the time she has the words to ask, she's already been depleted for a week.

This is the opposite. You don't need to wait for instructions. You don't need to ask "what can I do?" You don't need to be told three times.

You need a playbook. This is it.

Read it before the baby comes. Re-read week one. Reference week-by-week as you go. Highlight the parts you'll forget. Hand sections to your parents if needed.

You're going to do great. You're also going to mess some of this up. Both are fine. The point is to start.

— Soothemade

This isn't medical advice. If something seems wrong with your partner or the baby, call your provider. The US Maternal Mental Health Hotline is 1-833-852-6262 if she ever sounds like she's drowning — or if you are.



The one thing to remember

If you remember nothing else: **she is recovering from something the size of a major surgery, with no sleep, while keeping a human alive with her body.** Everything else flows from holding that truth steady.

You're not "helping." You're co-parenting and running the household. There's a difference, and she can feel it.



The first 48 hours (hospital + homecoming)

Do these things — no asking

- Take photos of everything. The first feed. The first diaper. Her holding the baby. Without showing anyone else yet. Just for you two.
- Be the gatekeeper. If a visitor (yes, even your mother) wants to come, you say no. You. Not her. If she wants to override you, she will.
- Write down everything the nurses say. Discharge instructions get fuzzy fast.
- Pack the bag. Drive the car. Carry the baby. She walks slowly — match her.
- Once home: don't put her in charge of anything yet. Not even the laundry. Especially not the laundry.

What you'll be tempted to do (don't)

-  Ask "what do you need?" 17 times — she doesn't have answers. Just do.
-  "Let her sleep" while announcing the news on social media. Ask her permission first.
-  Hand her the baby every time the baby cries. Take some of those cries yourself.
-  Eat all the freezer meals in week 1. Pace yourself — those are for week 3 when she's at her most depleted.



Week 1 — Survival mode

The list, in order of priority

1. Food and water

Every 2 hours, you bring her a glass of water and something to eat — without asking. A nut bar, half a sandwich, leftover pasta. If she's nursing, double everything.

Keep snacks within arm's reach of every place she sits or lies down. Bedside table. Nursing chair. Couch.

If she says "I'm not hungry" — that means "I haven't eaten and I don't have the energy to figure out food." Bring her a plate anyway. Put it in her hands.

2. The diaper / feed / sleep stack

Take every diaper change you can. Especially the night ones. She does not need to be awake for a diaper change. You can change a diaper. So can your father. So could literally anyone with hands.

If she's breastfeeding, you can't take the night feeds — but you can: - Bring the baby to her - Take the baby after the feed - Burp + diaper change + put the baby back down - Bring her water + a snack - Then you both go back to sleep

If you're bottle-feeding, you take half the nights. Not "you help" — you take them. Plan the rotation in advance.

3. The bleeding situation

She is bleeding for weeks. The pad situation is not glamorous. Restock the bathroom every day without being asked. Disposable mesh underwear is a gift from god — keep them in stock. Throw away the giant ice-pack diapers in a non-passive-aggressive way.

Do not ask her about it. Do not joke about it. Do not say "are you still bleeding?" Just keep the supplies stocked.

4. The shower

She needs to shower. You watch the baby for as long as it takes. If the baby cries, the baby cries. The baby is okay. You are okay. She needs the shower more than the baby needs the not-crying.

15 minutes of hot water and no one needing anything is the most important medicine in week 1.

5. The visitors

This is your job. Period.

If someone says "I'm coming over" — your response is "Not this week, we're sleeping. We'll text you in [X] when we're ready." You absorb their disappointment. Not her.

If your parents push back, hold the line. You are running interference. This is what partnership means right now.

If she changes her mind and wants a visitor — fine. But the default is no.

What success looks like for week 1

- She eats 3+ times a day.
- She drinks 8+ glasses of water a day.
- She sleeps in 2–3 hour stretches at minimum.
- She showers every day or two.
- She doesn't have to ask you for anything.
- You eat. You sleep when you can. You don't burn out by Friday.



Week 2 — The reality settles in

This is when the adrenaline starts to wear off and reality lands. She might cry more. She might be quieter. She might ask if “this is forever.”

She’s also possibly more functional during the day. That’s both good and a trap.

The check-in you owe her — once a day

A short, quiet question:

“What was the hardest moment today?”

Not “are you okay?” — too easy to deflect with “I’m fine.” A specific question gets a specific answer. She might cry. Sit with her. Don’t problem-solve unless asked.

The mental-health watch

Around day 10–14, baby blues should be easing. If she’s: - crying more, not less - saying “I don’t feel anything for the baby” - saying “you’d be better off without me” - not sleeping when the baby sleeps because she can’t stop checking - talking about scary intrusive thoughts she “can’t get rid of”

...this might be postpartum depression or postpartum anxiety. It’s common (1 in 7 birthing parents). It’s treatable. You do NOT wait for her to bring it up.

You say: “I think we should call your OB this week. I’ll dial. You’ll do great talking to them. Is that okay?”

If she says no, ask again in 24 hours. If she’s saying anything that sounds like she might hurt herself or the baby, you call her provider yourself, today.

(US Maternal Mental Health Hotline: 1-833-852-6262 — 24/7, free, confidential.)

What success looks like for week 2

- She's eating.
- She's hydrated.
- She's slept at least one 4-hour stretch.
- She's gotten out of the house at least once.
- You've checked in once a day with the specific question.



Week 3 — The “I should be fine by now” trap

Around week 3, the casseroles stop, the texts taper off, and your partner starts thinking she should be “back to normal.”

She is not back to normal. Her hormones just took the second drop. Her milk supply (if BF) is still regulating. She might be sleeping less, not more — because the baby’s sleep is changing.

This week is when most postpartum partners check out of “intense” mode and assume things are stable. Don’t.

The actions

- Keep cooking. Keep stocking snacks. The casserole-people stopped. The need didn’t.
- Take a longer chunk of baby care so she can sleep 4+ hours straight. Once this week. Even one good sleep changes everything.
- Plan a 2-hour solo errand for her. Coffee shop. Drive nowhere. Sit in a Target parking lot. She decides. You take the baby.
- Ask once: “Have you had time to think about anything that isn’t the baby this week?” If no, that’s a signal.



Weeks 4–6 — Re-entry

By now she's leaving the house more. She might be doing some work-from-home stuff. Things look more normal. This is when partners start backing off. Don't.

What's actually happening

- 6-week postpartum appointment is around now — go with her if at all possible. Take notes.
- If she's been cleared for activity / sex, do not assume that means she wants either yet. Wait for her cue.
- Postpartum identity grief peaks around now. "I don't recognize myself" is a real thing. Don't try to fix it. Listen.
- Hair starts falling out somewhere here. This is normal. Don't comment on it. Restock her vacuum.

The actions

- Keep doing all the household stuff. Do not "give it back to her."
- One day a week, you take primary baby duty for a chunk of hours. Even on weekdays if you can swing it.
- Schedule one date-like thing — coffee at home, a movie after the baby's down, a walk. Doesn't need to be fancy. Just signals "I see you, not just the parent-version of you."



Weeks 7–12 — The “fourth trimester”

The baby is sleeping in longer stretches. You’re both more functional. This is when the deeper postpartum stuff often surfaces — once survival mode ends and there’s space to feel.

Watch for

- Delayed PPD onset (can show up 2–4 months postpartum, not just week 1)
- Rage — postpartum rage is real and underdiscussed. If she’s snapping at you, the dog, or strangers more than seems normal, gently mention it
- Relationship friction — your communication has probably degraded. Schedule a real talk

The actions

- Have the “how are we doing?” conversation. Not about the baby — about you two. 30 minutes, no phone, no baby (or with baby napping).
- Take a Saturday and let her have the full day. You + baby out. She at home alone or with a friend.
- If she’s headed back to work, your role shifts again — see “Back to work” below.



Special situations

If she had a C-section

- She can't lift more than the baby for 6 weeks. You do all lifting. Period. No "she said she could grab the laundry basket." She's not supposed to.
- Stairs are limited for the first 1–2 weeks. Set up a downstairs station with everything she needs for the day.
- Driving is restricted (usually 2 weeks, sometimes longer). You drive. Or arrange someone else to.
- The scar will itch. Don't joke. Refill the witch hazel pads.

If she's breastfeeding

- You can't take the feeds. You can take everything else around them.
- Around 3am during a 45-minute cluster feed, sit with her. Don't fall asleep. Talk. Or just be present. Loneliness at 3am hits hardest.
- If she's pumping, you handle the bottle cleaning. All of it. Every part. Sterilizing supplies, drying rack, the works.

If she's formula feeding

- You take half the night feeds. Not "you help" — half. Set up the rotation in advance: which nights are yours, which are hers.
- The bottle prep station is yours to maintain. Clean and stocked, always.

If she had a difficult delivery / trauma

- She will replay it. Don't try to fix it. Listen. "That sounds really hard. I'm sorry I couldn't take that for you."

- Birth trauma can manifest weeks later. If she has flashbacks, intrusive thoughts, or won't talk about the birth — gently raise therapy. Not "you should see someone." Try: "I've been thinking about how hard the birth was. Would you want to talk to someone about it? I can find someone if it helps."

If you're a same-sex couple / non-birthing parent

- All of the above applies. None of this is gendered.
- One extra note: your partner may feel that her body's experience is "her thing" and yours isn't. Don't internalize that. You're a parent. Take the diapers, the baths, the night wakes, the everything. Your bond with the baby comes from the doing.

If she's gone back to work

- The mental load is now back on her in new ways. You track daycare bag, doctor appointments, the family calendar.
- Pumping at work — you stock the cooler, wash the parts every night, drive her to work if she's leaking.
- Drop-off transitions are emotional. Take half of them if your schedule allows. Even just for the first month back.



The conversation to have *now* (before baby comes)

Pull up this guide. Read it together. Then talk through these:

1. **The visitor rule.** What's our default for the first 4 weeks? Who tells people?
2. **The night plan.** Who takes which feeds / changes? Decide in advance.
3. **The "I'm not okay" word.** What word or phrase will she say if she's struggling and needs you to do something, not just listen?
4. **The "I need a break" plan.** When she says "I need to leave for 2 hours," your default is yes. Where does she go? You're not invited.
5. **The mental-health check-in.** Will you have it daily? Every other day? What's the actual sentence you'll say?
6. **The "you should call someone" trigger.** Agree in advance: if she's [X, Y, Z], you call her provider whether she wants to or not.
7. **The chores.** Make a list of all household tasks. Mark which one of you owns each, postpartum, until further notice.

Do this conversation 2–4 weeks before her due date. Re-do it at week 3 postpartum.



What you'll want to hear, but won't

- "Thank you for everything." She might not say it for weeks. Don't take it personally. She's busy keeping a human alive.
- "I appreciate you." Same.
- "Sorry I snapped at you." She might. She might not. Either is okay.

Your reward is not her praise. Your reward is the relationship you have with your kid in 5, 10, 20 years — and the relationship with her, intact, on the other side of this season.



A note for you

The non-birthing partner mental-health gap is real. Postpartum depression in fathers/partners affects about 10%. You're not just a support system — you're also navigating becoming a parent. You're also sleep-deprived. You're also possibly working full-time on no sleep.

Things you can do for yourself, without guilt:

- 30 minutes of exercise a day, even on bad days. Walks count.
- Tell one friend what's actually going on. Not the sanitized version.
- Stay off Instagram if it's making you feel bad. Other parents look like they're crushing it. They're not. The post is from their best 8 seconds of the week.
- If you feel detached, anxious, hopeless, or numb — call a provider. The hotline above works for you too.

You're allowed to be tired. You're allowed to be sad. You're allowed to find this hard.

You are doing one of the most important things a human can do.



The summary card (clip and keep)

Daily: - Bring food + water without asking - Take a turn with the baby longer than you think - Ask the specific check-in question once

Weekly: - Plan one solo break for her (2+ hours) - Stock supplies (pads, snacks, water) - Take primary baby duty for a chunk - Do not “give household back” — keep owning it

Beyond week 4: - Mental-health watch never ends — watch for delayed PPD/PPA - Re-have the chores + visitor + mental-health conversation - Be the planner for her postpartum appointments

The line that should never go away:

“What was the hardest moment today?”

You’ve got this.

— Soothemade



This is a planning guide for partners of birthing people, written from general experience. Not medical, psychological, or legal advice. For questions about your specific situation, contact your healthcare provider. If your partner is in crisis, call the US Maternal Mental Health Hotline at 1-833-852-6262, or 988 (Suicide & Crisis Lifeline), or 911.



Soothemade *Notes*

*Made for the version of you no one tells you about — the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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